

STRAIGHT PEPTIDES

TRUE IN HEALTH. UNLOCKING HUMAN POTENTIAL

PRODUCT GUIDE

Retatrutide 20mg — Mixing & Injection Instructions

Reconstitution and Subcutaneous Administration Protocol For Straight Peptides Clients

Confidential | Internal Use Only | 2026

Stage 1: Reconstitution

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Mixing Your Retatrutide Vial

Time: 7 minutes | Location: Clean, flat surface

The 20mg vial requires two separate draws of bacteriostatic water. This double-draw process delivers a final concentration of 0.1mg per unit — matching the 10mg and 30mg vials so dosing stays consistent across the line.

Products Needed

Bacteriostatic Water	BAC water vial (supplied)
1mL / 100-unit insulin syringe	For drawing and transferring BAC water
Alcohol swabs	For sterilizing vial stoppers and injection site
20mg Retatrutide vial	Lyophilized peptide, vacuum-sealed

Prepare the Vials

Process

1. Remove the protective seals from both the BAC water and Retatrutide vials.
2. Clean the rubber stoppers on both vials using an alcohol swab. Allow to air dry.

Mixing Instructions

Protocol

1. Using the 1mL insulin syringe, insert the needle into the center of the BAC water vial.
2. Push air into the vial, then pull back to 100 units (1mL) of BAC water.
3. Insert the needle into the Retatrutide 20mg vial.

Critical Warning

Do not push the syringe. The vial is vacuum-sealed and draws the liquid in on its own. Forcing the plunger compromises the vacuum and may cause inaccurate dosing.

1. Repeat steps 1–3 once more, for a total of 2mL (200 units) of BAC water in the vial.

Protocol Note

The 20mg vial requires two full draws of 100 units each (200 units total). Do not attempt to shortcut this process. Each draw uses a fresh insertion into the BAC water vial. The final concentration is 0.1mg per unit.

1. Gently roll the vial between your fingers. Do not shake. Rotate slowly for 30–60 seconds until fully dissolved.
2. Place the vial in the refrigerator. After 2 hours refrigerated, the solution is ready for injection.

Storage Requirement

Reconstituted Retatrutide must remain refrigerated at all times. Use within 30 days of reconstitution for best potency. Do not freeze or expose to direct sunlight or heat.

Dose Conversion

20mg vial mixed with 2mL BAC water = 0.1mg per unit

20 units	2mg
40 units	4mg
60 units	6mg
80 units	8mg
100 units	10mg
120 units	12mg

Vial Duration

2mg weekly	10 weeks
4mg weekly	5 weeks
6mg weekly	≈3–4 weeks
8mg weekly	≈2.5 weeks
10mg weekly	2 weeks
12mg weekly	≈1.5 weeks

Stage 2: Injection

2

Subcutaneous Administration

Time: 2 minutes | Location: Stomach, outer thigh, or upper glute

Administer your dose subcutaneously using an insulin syringe. Rotate injection sites weekly to prevent lipodystrophy.

Products Needed

Retatrutide vial	Reconstituted and refrigerated
Alcohol swab	For sterilizing vial stopper and injection site
Insulin syringe	1mL / 100-unit, for drawing and injecting the dose

Draw Your Dose

Protocol

1. Wipe the Retatrutide vial stopper and your injection site with an alcohol swab.
2. Insert the needle into the center of the rubber stopper.
3. Pull back to your prescribed dose on the syringe using the conversion table above.

Inject

Process

1. Insert the needle fully and smoothly into the injection site at a 90-degree angle. Preferred sites: stomach, outer thigh, or upper glute.
2. Inject slowly and steadily.
3. After injecting, gently rub the area in a circular motion to aid absorption.
4. Dispose of the syringe safely.

Stage 3: Dosing Protocol

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Frequency, Titration, and Storage

Time: Ongoing | Once weekly

Retatrutide is administered once weekly on the same day each week. Gradual titration is critical for tolerability.

Important Reminders

Protocol

Refrigeration	Always keep reconstituted Retatrutide refrigerated
Sterilization	Always wipe the vial stopper and injection site before each use

Shelf life	Use within 30 days of reconstitution for best potency
Frequency	Inject once weekly, same day each week
Injection sites	Rotate sites to prevent lipodystrophy

Titration

Objective

Start at the lowest effective dose and titrate up gradually every 2–4 weeks as tolerated. This minimizes GI side effects and allows the body to adapt to the triple-agonist mechanism.

Protocol Note

Retatrutide activates GIP, GLP-1, and glucagon receptors simultaneously. Starting low and titrating slowly reduces nausea, fatigue, and lightheadedness that commonly occur at higher doses. Do not jump doses.

Stage 4: What to Expect

4	Response Timeline and Outcomes Timeline: Weeks 1–12+
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Retatrutide is a triple agonist targeting GIP, GLP-1, and glucagon receptors, making it one of the most potent body composition compounds currently in research. It works through multiple pathways to reduce appetite, increase energy expenditure, and promote fat loss.

Early Changes

Weeks 1–4

Appetite	Reduced appetite and food cravings
Satiety	Feeling full after smaller meals
Nausea	Mild, especially at higher doses — typically subsides as the body adjusts
Energy	Mild fatigue or lightheadedness early in titration

Progressive Results

Weeks 4–12+

Body fat	Significant reduction including visceral fat
Glucose and insulin	Improved fasting glucose and insulin sensitivity
Lean mass	Preservation of lean muscle mass during fat loss
Metabolic rate	Enhanced via glucagon receptor activity

Overall composition	Improved body composition and energy levels
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Important Notice

This guide is intended for research and educational purposes only. Not intended to diagnose, treat, cure, or prevent any disease.

Questions or Concerns

Contact Straight Peptides directly for dosage guidance, product questions, or support.

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